

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

August 28, 2020

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
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Chief, Investigations Unit
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FROM: CDR (b)(6),(b)(7)(C),(b)(7)(F), RN (b)(6),(b)(7)(C),(b)(7)(F)
Investigator
ICE Health Service Corps

SUBJECT: Mortality Review – Report of Findings
Ramiro HERNANDEZ Ibarra, (b)(6),(b)(7)(C)

Executive Summary:

On March 21, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Ramiro HERNANDEZ Ibarra. Mr. HERNANDEZ, a 42-year-old Mexican male, was in ICE custody from January 15 – March 21, 2020, and assigned to Port Isabel Detention Center (PIDC) in Los Fresnos, Texas (TX), on the date of his death.

Prior to ICE custody, on December 20, 2019, Hidalgo County Sheriff's Office arrested Mr. HERNANDEZ on charges of assault. On December 21, 2019, Enforcement and Removal Operations (ERO) San Antonio encountered Mr. HERNANDEZ at the Hidalgo County Jail

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(HCJ) in Edinburg, TX, served him a Notice to Appear, form I-862, charging removability as an alien, present without admission or parole by an immigration officer. On January 15, 2020, HCJ released Mr. HERNADEZ to ERO San Antonio. ERO transferred Mr. HERNADEZ to PIDC for housing.

During Mr. HERNADEZ's custody at PIDC, he reported a history of diabetes mellitus [(DM); a chronic condition characterized by high blood sugar, because the body does not produce enough insulin to meet its needs] and hypertension [(HTN); a chronic condition characterized by high blood pressure]. Mr. HERNADEZ reported taking medications and denied any mental health conditions, or suicidal history. On January 16, 2020, an advanced practice provider (APP) completed Mr. HERNADEZ's physical examination, ordered his DM and HTN medications, laboratory studies, and an electrocardiogram [(EKG); measures the electrical activity of the heart].

On March 18, 2020, at approximately 11:14 p.m., Mr. HERNADEZ's housing officer requested medical assistance over the radio, after he witnessed Mr. HERNADEZ's multiple episodes of vomiting. The shift captain (Capt.), custody officers (COs), two registered nurses (RN), and a licensed vocational nurse (LVN) arrived on the scene, assessed Mr. HERNADEZ, and transported him to the medical clinic for further evaluation. Because Mr. HERNADEZ continued with nausea, vomiting, and diarrhea, the medical staff notified the on-call APP, who gave a verbal order to place Mr. HERNADEZ in the medical housing unit (MHU) for observation. At approximately 5:25 a.m., a control center officer radioed for medical assistance after he witnessed, via video surveillance, Mr. HERNADEZ fall in his cell and hit his head on the sink and wall. Shortly thereafter, several nursing staff and COs arrived to Mr. HERNADEZ's cell. Mr. HERNADEZ reported having flank pain and requested ibuprofen. The RN assessed Mr. HERNADEZ, did not observe any injuries, and called the on-call APP, who gave a verbal order to transfer Mr. HERNADEZ to Valley Baptist Medical Center (VBMC) in Harlingen, TX, for further evaluation. Emergency medical services (EMS) arrived at approximately 5:52 a.m., observed Mr. HERNADEZ as alert and oriented, with normal respirations, flank and lower back pain, and transported him to VBMC.

A VBMC medical doctor (MD) evaluated Mr. HERNADEZ, diagnosed him with nausea, vomiting, shortness of breath, and altered mental status, requiring subsequent admission. Mr. HERNADEZ became hypothermic (low temperature), hypotensive (low blood pressure), suffered metabolic acidosis (condition where too much acid accumulates in the body), electrolyte imbalance, and respiratory failure. The MD ordered emergent hemodialysis (a treatment to filter wastes and water from the body), subsequent dialysis therapy, intubation for Mr. HERNADEZ's impeding respiratory failure and confusion, and transferred him to the intensive care unit (ICU). Mr. HERNADEZ eventually suffered multi-organ failure and became unresponsive. VBMC suspected possible septic shock (widespread infection causing organ failure) versus multifactorial hypovolemic shock (a medical emergency in which blood volume drops to a dangerous level) and suspected his condition to be related to metformin (medication

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used to treat diabetes) overdose. On March 21, 2020, Mr. HERNANDEZ's condition deteriorated, and he no longer tolerated dialysis treatments. At approximately 5:46 a.m., Mr. HERNANDEZ expired while on ventilatory support, and a VBMC medical doctor (MD) pronounced him deceased. An autopsy was requested, but not completed, because IHSC could not locate a medical examiner who would accept the case and perform the exam.

Mortality Finding:

As a result of Mr. HERNANDEZ's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) unanimously determined that Mr. HERNANDEZ's medical care at PIDC was not provided within the safe limits of practice. Specifically, the MRC determined PIDC's care or lack thereof, could pose a significant future risk. Overall findings identified were: 1) PIDC nursing staff did not complete nursing assessments to document Mr. HERNANDEZ's symptoms, 2) poor documentation of the care provided to Mr. HERNANDEZ, and 3) the nursing staff failed to refer Mr. HERNANDEZ to a higher level of care for his worsening symptoms.

The MRC convened on July 16, 2020, and consisted of the following members:

(b)(6),(b)(7)(C),(b)(7)(F) Doctor of Osteopathic Medicine (DO), Houston Detention Center's Clinical Director, physician reviewer; Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) Physician Assistant - certified, advanced practice provider (APP) reviewer; Lieutenant (LT) (b)(6),(b)(7)(C),(b)(7)(F) Health Care Risk Management Program Manager, risk management reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) registered nurse (RN), Chief Nurse, nurse reviewer; and Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) Behavioral Health Consultant, mental health reviewer.

The following summary of events was based on: 1) Mr. HERNANDEZ's PIDC and VBMC medical records, and EMS reports; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. HERNANDEZ's PIDC detention file; 6) video footage from March 18, 2020; 7) a remote review and staff interviews conducted by fact-finder CDR (b)(6),(b)(7)(C),(b)(7)(F) and IHSC's Behavioral Health Consultant, CDR (b)(6),(b)(7)(C),(b)(7)(F) on May 4 - 7, 2020; and 8) applicable IHSC policies and procedures, and ICE Detention Standards.

Summary of Events

On January 15, 2020, an LVN completed Mr. HERNANDEZ's medical intake screening. Mr. HERNANDEZ reported a history of DM and HTN, controlled with medications, but could not

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recall the medication names. He denied pain, discomfort, mental health illness or history of suicide. His vital signs (VS) were normal except for an elevated blood pressure (BP) of 145/91 millimeters of mercury [(mmHg); normal range 90/60 – 120/80] and an elevated blood sugar (BS) of 175 milligrams/deciliter [(mg/dl); normal range 70 – 100 mg/dl].

On January 16, 2020, an APP completed Mr. HERNANDEZ's physical examination. Mr. HERNANDEZ reported a five-year history of DM, HTN, and taking metformin 1000 mg, one tablet by mouth, two times daily. Mr. HERNANDEZ did not recall the name of his HTN medication(s). The APP ordered Mr. HERNANDEZ the following medications: metformin 1000 mg, one tablet by mouth, two times daily, lisinopril (anti-hypertensive medication) 20 mg daily, ammonium lactate cream for dry skin, an EKG, and laboratory studies for DM and HTN.

On February 7, 2020, an APP completed Mr. HERNANDEZ's chronic care evaluation, reviewed his laboratory results, that showed normal results, except for elevated lipids (triglycerides - 341 mg/dl; normal level less than 150 mg/dl), glucose (123 mg/dl), hemoglobin A1C (9.4 percent) [(%); the average level of blood sugar over the past two to three months; normal level is less than 6.0 %], and trace protein and ketones (by-products of the breakdown of fatty acids) in his urine. Mr. HERNANDEZ complained of left flank (area on side of the body between abdomen and the back) pain, reported past kidney issues, and decreased water intake. The APP prescribed Mr. HERNANDEZ atorvastatin (medication used to treat elevated cholesterol and triglyceride levels) 10 mg, one tablet by mouth, daily; Pedialyte (rehydration mixture for treatment of dehydration); and instructed him to increase his fluid intake.

On February 7, 9, 12, 15, and 16, 2020, Mr. HERNANDEZ refused his lisinopril and metformin medications. The nursing staff notified the APP of Mr. HERNANDEZ's medication non-compliance. An APP educated Mr. HERNANDEZ on February 7, 2020, on the importance of his medication compliance; however, there is no documentary evidence that a medical provider counseled Mr. HERNANDEZ after that date.

Between February 13 – 17, 2020, an RN and APP evaluated Mr. HERNANDEZ for acute left flank pain, radiating to his lower back. On February 17, 2020, Mr. HERNANDEZ reported having pain with ambulation, movement, and no relief with pain medication. An RN noted Mr. HERNANDEZ's difficulty with ambulation, notified the on-call provider, and received a verbal order to transfer Mr. HERNANDEZ to VBMC emergency department (ED), via EMS for evaluation.

On February 18, 2020, a VBMC MD evaluated and diagnosed Mr. HERNANDEZ with low back pain and a urinary tract infection (UTI).

- At approximately 1:52 a.m., the MD discharged Mr. HERNANDEZ with the following medications: cephalexin (antibiotic), cyclobenzaprine (muscle relaxer), and ibuprofen (pain reliever).

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- At approximately 4:00 a.m., Mr. HERNADEZ returned to PIDC and an RN evaluated him, reviewed his VBMC discharge summary, and contacted the on-call provider. An APP gave a verbal order to start sulfamethoxazole/trimethoprim (antibiotic), ibuprofen, and to house Mr. HERNADEZ in the MHU for observation.
- At approximately 2:26 p.m., an APP evaluated Mr. HERNADEZ in the MHU. Mr. HERNADEZ requested to return to the dorm with keep-on-person (KOP) medications versus direct observation on pill line, due to his difficulty with ambulation. He denied having back pain, dysuria (painful urination), hematuria (blood in urine), chills, or sweats. The APP discharged Mr. HERNADEZ from the MHU with KOP medications, despite Mr. HERNADEZ's previous history of non-compliance to medications.

On February 24, 2020, an APP completed Mr. HERNADEZ's post MHU discharge follow up evaluation. Mr. HERNADEZ reported having lower back pain, due to cold weather. The APP reviewed Mr. HERNADEZ's VBMC laboratory and imaging studies, documented Mr. HERNADEZ's resolved UTI, normal gait, full range of motion, and lower back tenderness with palpation to area. The APP ordered Mr. HERNADEZ tramadol (pain reliever) 50 mg, two times daily, as needed, encouraged him to increase his water intake, and authorized a lower bunk with an extra mattress.

On March 18, 2020, at approximately 11:14 p.m., Mr. HERNADEZ's housing officer requested medical assistance over the radio after he witnessed Mr. HERNADEZ's multiple episodes of vomiting at his bedside and in the bathroom. The shift Capt., COs, two RNs, and an LVN arrived on the scene. Another detainee reported Mr. HERNADEZ swallowed some pills and gave the officer a note, which contained two names, telephone numbers, and Mr. HERNADEZ's apology. *[Investigator's note: none of the COs were able to identify the detainee or acknowledge receiving the note.]* The housing officer informed the medical staff that Mr. HERNADEZ allegedly ingested several pills. The medical staff assessed Mr. HERNADEZ in the dorm, witnessed an episode of vomiting, and transported him to the medical clinic, via wheelchair.

- At approximately 11:35 p.m., an RN evaluated Mr. HERNADEZ in the urgent care room. A CO gave Mr. HERNADEZ's bag of medications to the RN and the RN observed an empty metformin bottle, but Mr. HERNADEZ denied taking any medications. Mr. HERNADEZ reported non-compliance with his DM medication for a few days and last meal of Hot Cheetos with subsequent diarrhea (five bowel movements). Mr. HERNADEZ denied having any other symptoms. The RN noted normal VS, except for a slightly elevated heart rate of 109 beats per minute [(bpm); normal 60-100 bpm] and an elevated blood sugar of 205 mg/dl. The RN notified the on-call APP, received a verbal order to admit Mr. HERNADEZ into the MHU, administer oral and intramuscular medications to treat his nausea and vomiting, and encourage oral hydration. *[Investigator's note: The RN did not report the possible overdose to the on-call APP.]*

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On March 19, 2020, PIDC's MHU video surveillance footage showed an RN transferred Mr. HERNADEZ from the urgent care room to the MHU via wheelchair. Mr. HERNADEZ independently transferred himself from the wheelchair to the bed. While in the bed, Mr. HERNADEZ presented as restless, alternately moving from the head to the foot of the bed.

- At approximately 4:58 a.m., Mr. HERNADEZ stumbled to the toilet with an unsteady gait, lost his balance, and placed his hands on the floor to prevent falling. As he stood up from the toilet with an unsteady gait, he stumbled back to his bed with his pants around his ankles and pulled up his pants after lying on the bed.
- At approximately 5:08 a.m., Mr. HERNADEZ attempted to ambulate to the toilet but stumbled and fell to the ground. Approximately thirty seconds later, a CO entered the room, noticed Mr. HERNADEZ lying on the floor, briefly spoke to him, and Mr. HERNADEZ crawled back to the bed. Shortly thereafter, an RN came to the doorway but did not assess Mr. HERNADEZ.
- At approximately 5:17 a.m., an RN administered Mr. HERNADEZ ibuprofen 400 mg, by mouth for flank and back pain. Throughout the night, Mr. HERNADEZ ambulated multiple times to the toilet in his cell.
- At approximately 5:26 a.m., Mr. HERNADEZ ambulated to the toilet again with an unsteady gait and fell to the floor. After a few minutes, Mr. HERNADEZ sat up, dazed, and crawled back to the bed.
- At approximately 5:28 a.m., three nurses and a CO arrived to Mr. HERNADEZ's cell, attempted to obtain his VS, but Mr. HERNADEZ presented confused, restless, and constantly moving his extremities.
- At approximately 5:37 a.m., during video surveillance monitoring, a control center officer witnessed Mr. HERNADEZ fall and hit his head on the wall and sink, and radioed for medical assistance. A few seconds later, medical staff and COs arrived. Mr. HERNADEZ reported 9 out of 10 [(normal range 0 (pain free) - 10 (severe pain))] flank pain and requested ibuprofen. The RN documented no injuries; however, review of the video surveillance footage did not indicate the RN completed Mr. HERNADEZ's assessment. After COs assisted Mr. HERNADEZ up from the ground, Mr. HERNADEZ ambulated a couple of steps, then fell backwards. The COs assisted him to the floor, and the nursing staff attempted to obtain his vital signs, but to no avail, due to Mr. HERNADEZ's continued movement of his extremities. The COs assisted Mr. HERNADEZ to the bed.
- An RN notified the on-call APP and received a verbal order to transfer Mr. HERNADEZ to VBMC for further evaluation.

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- At approximately 5:45 a.m., EMS arrived, observed Mr. HERNADEZ alert oriented, with normal respirations, but with flank and lower back pain.
- At approximately 5:56 a.m., EMS departed PIDC.
- At approximately 6:49 a.m., VBMC ED personnel evaluated Mr. HERNADEZ, ordered laboratory and radiology studies, which showed Mr. HERNADEZ had severely impaired kidney function and severe metabolic acidosis. His VS were normal, except for a low BP of 81/44 mmHg.
- At approximately 8:10 a.m., a PIDC RN informed the VBMC ED medical staff of Mr. HERNADEZ's potential ingestion of "a lot" of pills, but did not provide the names or number of pills ingested. However, the RN provided Mr. HERNADEZ's KOP medication list and noted empty bottles of metformin, lisinopril, and ibuprofen.
- At approximately 9:36 a.m., while in the ED, Mr. HERNADEZ's condition worsened, and he developed altered mental status, hypotension (92/61 mmHg), hypothermia (rectal temperature of 88.6 degrees (°) Fahrenheit [(F); normal range is 98.6-100.6°F], and shortness of breath, requiring ICU admission and intubation, due to impending respiratory failure and confusion. The MD ordered antibiotics, vasopressors (medications used to stabilize low blood pressure levels), and a nephrology consult.
- At approximately 6:31 p.m., a nephrologist diagnosed Mr. HERNADEZ with kidney failure, severe metabolic acidosis, and ordered hemodialysis.

On March 20, 2020, at approximately 1:15 a.m., Mr. HERNADEZ's condition improved and his BP readings increased within normal limits; however, he remained on antibiotics, epinephrine (for blood pressure support), and continued dialysis.

- At approximately, 5:15 a.m., Mr. HERNADEZ's condition worsened.
- At approximately 9:15 a.m., Mr. HERNADEZ became unresponsive, in kidney failure, and in critical condition with imminent death. The VBMC medical staff contributed Mr. HERNADEZ's condition as a result of metformin overdose.
- At approximately 4:20 p.m., a VBMC MD informed IHSC of Mr. HERNADEZ's multi-organ failure and unresponsiveness.

On March 21, 2020, at approximately 12:30 a.m., Mr. HERNADEZ continued to decompensate, and he no longer tolerated his dialysis treatments.

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- At approximately 5:46 a.m., Mr. HERNADEZ expired while on ventilatory support, and a VBMC MD pronounced him dead.

Mr. HERNADEZ's death certificate showed the cause of death was due to multi-organ failure related to metformin overdose. An autopsy was requested, but not completed, because IHSC could not locate a medical examiner who would accept the case and perform the exam.

CDR (b)(6),(b)(7)(C) Behavioral Health Consultant, performed Mr. HERNADEZ's psychological autopsy (procedure for analyzing the cause of death by reconstructing what the person thought, felt, and did before death, based on gathered information). The final psychological autopsy report is pending.

ICE detention standards used for this review: ICE Performance-based National Detention Standards (PBNDS) 2011. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) PIDC custody staff timely activated a medical emergency notification; 2) PIDC's medical staff responded to the medical emergency within four minutes and transferred Mr. HERNADEZ to medical for evaluation; 3) the PIDC medical staff appropriately housed Mr. HERNADEZ in the MHU for observation; and 4) PIDC medical staff timely activated EMS after responding to the second medical emergency.

Weaknesses

1. Timely referrals.

- On March 19, 2020, while in the MHU, Mr. HERNADEZ fell three times and developed additional symptoms of flank and back pain; however, the nursing staff did not report the change to the on-call provider, resulting in approximately a 37-minute delay in referral to higher level of care.

Applicable standards of care for this finding:

- ICE PBNDS 2011, Medical Care., section T. *Emergency Medical Services and First Aid.*
- IHSC Operations Memorandum (OM), 16-027, *Clinical Nursing Guidelines (Interim RN Guidelines).*

2. Patient assessments.

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- On March 18, 2020, Mr. HERNADEZ experienced severe vomiting and diarrhea, but the medical record did not include any documentary evidence to indicate PIDC's nursing staff completed an assessment addressing his clinical complaint.
- On March 19, 2020, Mr. HERNADEZ fell three times while in the MHU, but there is no evidence the nursing staff completed an assessment or referred him to a higher-level provider after the first two falls.
- On March 19, 2020, Mr. HERNADEZ fell a third time, and hit his head, but the nursing staff did not complete a neurological assessment.
- While in the MHU, Mr. HERNADEZ complained of flank pain, but nursing staff did not complete a pain assessment.
- Mr. HERNADEZ refused his HTN and DM medication on multiple occasions. An APP counseled Mr. HERNADEZ after the first refusal, but there is no documentary evidence a provider evaluated, educated, or counseled Mr. HERNADEZ after his subsequent medication refusals.

Applicable standards of care for this finding:

- IHSC OM, 16-027, *Clinical Nursing Guidelines (Interim RN Guidelines)*.
- IHSC Directive, 02-08, *Refusal of treatment*.
- National Commission on Correctional Health Care (NCCHC), J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.

3. Documentation.

- On March 18, 2020, the nursing staff notified the on-call APP; however, there is no documentation in the medical record to indicate what information they reported to the APP.
- On March 18, 2020, Mr. HERNADEZ experienced severe vomiting and diarrhea; however, the nursing staff did not document a thorough description or clinical assessment of his symptoms.
- An RN administered ibuprofen to Mr. HERNADEZ while in the MHU but did not document a pain assessment level.
- On March 19, 2020, while in the MHU, Mr. HERNADEZ fell, and hit his head; however, the medical record did not include any evidence to indicate the RN completed a neurological, or physical assessment.

Applicable standards of care for this finding:

- IHSC OM, 16-027, *Clinical Nursing Guidelines (Interim RN Guidelines)*.
- IHSC OM, 16-022, *IHSC Health Records Management*.
- NCCHC, J-A-08, *Health Records*.

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4. Communication.

- On March 18, 2020, the nursing staff notified the on-call provider, but it is unclear what they communicated regarding the patient's symptoms or condition.
- While in the MHU, Mr. HERNADEZ complained of flank pain and fell three times; however, the nursing staff did not report a change in condition to the on-call provider.
- The medical staff did not report a potential head injury to EMS personnel.
- The nursing staff denied custody provided timely information regarding Mr. HERNADEZ's possible medication overdose.

Applicable standards of care for this finding:

- ICE PBNDS 2011, Medical Care., section BB. 4. *Transfer and Release of Detainees.*
- IHSC OM, 16-027, *Clinical Nursing Guidelines (Interim RN Guidelines).*
- ICE PBNDS 2011, Detainee Transfers., C. *Responsibilities of the Health Care Provider at the Sending Facility.*
- NCCHC, J-B-08, *Patient Safety.*
- NCCHC, J-D-08, *Hospital and Specialty Care.*
- NCCHC, J-E-09, *Continuity, Coordination, and Quality of Care during Incarceration.*

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, the PIDC administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the CAP is implemented and sustained.
- Based on the Chief Nurse's participation in the mortality review process, the Nursing Services Unit will further review the nurses' response and address through formal action with PIDC to reduce further risk.

End of report.